



Department of Microbiology & Pathology
Central Hospital, North Central Railway
Prayagraj.

Lab Registration No.

Name:- **SUDHA SRIVASTAVA**

Ward :- HDU-6

CR No:- 991922200205615

Regd Date :-06-09-2023

Reporting Date:- 06-09-2023

Age/Sex :- 64/M

Referred by :- Dr ANURAG

Dengue Test Report – Automated testing

TEST	RESULT	Unit	Result Interpretation
Dengue NS1	NEGATIVE	0.10	< 1.0 (Index Value) NEGATIVE
Dengue Ig M	NEGATIVE	0.04	≥ 1.0 (Index Value) POSITIVE

Method: Test performed with automated testing on ELFA technology (miniVIDAS/VIDAS) –
Interpretation of results in different stage of Infection :

Dengue infection stage	VIDAS® DENGUE NS1 Ag	VIDAS® Anti-DENGUE IgM	VIDAS® Anti-DENGUE IgG
Naïve	Negative	Negative	Negative
Acute infection	Positive	Positive	Negative
Post-acute infection	Negative	Positive	Positive
Recovery	Negative	Negative	Positive

Notes:

- Dengue virus (DENV) belongs to the Flaviviridae family and is transmitted to humans by Aedes mosquitoes. DENV are divided into four serotypes: DEN-1, DEN-2, DEN-3 and DEN-4. The test detects all 4 serotypes.
- Recommended test is NS1 Antigen in the first 5 days of fever. After 7-10 days of fever, the recommended test is Dengue fever antibodies IgG & IgM.
- Negative IgM result does not rule out Dengue infection. Additional sample to be tested after 7-14 days if infection is suspected, and if positive Suggestive of Primary / Secondary Dengue infection.
- Negative IgG indicates a presumptive evidence that the patient does not have secondary Dengue infection and if positive indicating presumptive evidence that the patient has been recently exposed to / or currently infected with Dengue virus
- In presence of detectable dengue NS1 antigen, Dengue IgG & IgM serology assays should be performed on follow up samples after 5-7 days of onset of fever, to confirm dengue infection.
- The disease may be subclinical, self limiting, febrile or may progress to a severe form of Dengue hemorrhagic fever or Dengue shock syndrome.
- Interference may be encountered with certain sera containing antibodies directed against reagent components. For this reason, assay results should be interpreted taking into consideration the patient's clinical history and the results of any other tests performed

[Signature]
CLS

CH/NCR/PRY



NORTH CENTRAL RAILWAY CENTRAL HOSPITAL

Nawab Yusuf Road

Prayagraj

Allahabad-211001, Uttar Pradesh, INDIA



Department of Laboratory Medicine

Hematology

CR No : 991922200205615 Lab/Study No. : 0907HMO34 Requisition Date : 07-Sep-2023 12:58
Patient Name : Sudha Srivastava Age/Sex : 64 Yr/F Coll./Study Date : 07-Sep-2023 13:44
Sample Type/No : Edta Whole Ward : Ward 4 Reporting Date : 07-Sep-2023 13:59
Blood/0709W6007
Room/Bed : Hdu/Hdu 6 Dept/Unit : General - GDMO R No 12
Investigation Result Unit Ref. Range

Complete Blood Picture

Hemoglobin	11.2	g/dl	12 - 15
Hematocrit (PCV)	35.8	%	36 - 46
Mean Corpuscular Volume (MCV)	79.3	fl	83 - 101
Mean Corpuscular Hemoglobin (MCH)	24.9	pg	27 - 32
Mean Corpuscular Hemoglobin Concentration (MCHC)	31.4	g/dl	31.5 - 34.5
Total RBC Count	4.51	10 ³ /mm ³	3.8 - 4.8
RDW CV	14.3	%	11.6 - 14
RDW SD	40.3	fl	39 - 46
Total WBC Count	13900	/mm ³	4000 - 10000
Neutrophils	87.4	%	40 - 80
Lymphocytes	6.2	%	20 - 40
Monocytes	6.2	%	2 - 8
Eosinophils	0.0	%	1 - 6
Basophils	0.2	%	0 - 2
Platelet Count	4.47	Lac/mm ³	1.5 - 4

Comments:

***** END OF THE REPORT *****

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Allahabad-211001,Uttar Pradesh, INDIA



Department of Laboratory Medicine

Biochemistry

CR No : 991922200205615 Lab/Study No. : 0907BI008 Requisition Date : 07-Sep-2023 10:31
Patient Name : Sudha Srivastava Age/Sex : 64 Yr/F Coll./Study Date : 07-Sep-2023 10:38
Sample Type/No : Serum/0709W4005 Ward : Ward 4 Reporting Date : 07-Sep-2023 12:49
Room/Bed : Hdu/Hdu 6 Dept/Unit : General - GDMO R No 12

Investigation	Result	Unit	Ref. Range
Lipid Profile:			
Serum Cholesterol	175.80	mg/dl	Desirable < 200; Boderline - 200-239; High > 239
Serum Triglycerides	76.43	mg/dl	Normal: < 150; High: 150-199; Hyper triglyceridemia:200-499; Very High > 499
Serum HDL Cholesterol	41.38	mg/dl	> 40 mg/dl
Serum LDL Cholesterol		mg/dl	Optimal: < 100; Near/above Optimal: 100- 129; Borderline : 130-159; High 160-189; Very High: > 189 mg/dl

Comments:

***** END OF THE REPORT *****

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Verified By: UMESH KR BHARTI, (LAB. SUPERINTENDENT(MEDICAL))

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Prayagraj

Allahabad-211001, Uttar Pradesh, INDIA



Department of Laboratory Medicine

Biochemistry

CR No : 991922200205615 Lab/Study No. : 0911B1191 Requisition Date : 10-Sep-2023 10:46
Patient Name : Sudha Srivastava Age/Sex : 64 Yr/F Coll./Study Date : 11-Sep-2023 13:19
Sample Type/No : Serum/1109W4009 Ward : IPD Reporting Date : 11-Sep-2023 18:17

Room/Bed : -/-	Dept/Unit : General - Gdmo R No 12		
Investigation	Result	Unit	Ref. Range
Renal Function Test			
Serum Creatinine	1.53	mg/dL	0.7 - 1.2
Serum Uric Acid	6.08	mg/dL	2.6 - 6.0

Comments:

***** END OF THE REPORT *****

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CENTRAL HOSPITAL
DEPARTMENT OF MICROBIOLOGY/MOLECULAR LAB
NORTH CENTRAL RAILWAY PRAYAGRAJ

Patient Name : SUDHA SRIVASTAVA	Received on : 11-09-2023
Age & Sex : 64/F	Reported on : 11-09-2023
Patient ID : 991922200205615	Ref. Doctor : Dr. Anurag Yadav
OPD/Ward : HDU-6	Serial No : 01/1109

Result

Test Name	Result	Units	Unicel DxI / Access 2 Reference Ranges
FT3 (Free Triiodo thyronine)	2.96	pg/mL	Range: 2.5 - 3.9
FT4 (Free Thyroxine)	1.58	ng/dl	Range: 0.61 - 1.12 Pregnancy: 1st Trimester 0.52 - 1.10 Pregnancy: 2nd Trimester 0.45 - 0.99 Pregnancy: 3rd Trimester 0.48 - 0.95
TSH (Thyroid Stimulating Hormone)	1.721	μIU/mL	Range: 0.34 - 5.6

The test has been carried out in Automated CLIA System-Beckman Coulter Access-2.

Interpretation

TSH	FT3	FT4	Interpretation
High	Normal	Normal	Subclinical Hypothyroidism
Low	Normal	Normal	Subclinical Hyperthyroidism
High	High	High	Secondary Hyperthyroidism
Low	High/Normal	High/Normal	Hyperthyroidism
Low	Low	Low	Non Thyroidal illness/Secondary Hypothyroidism

Comments

1. TSH, released from the anterior pituitary, is the principal regulator of thyroid function, stimulating the synthesis and release of thyroid hormones thyroxine (T4) and triiodothyronine (T3). T3 and T4 regulate biochemical processes that are essential for normal metabolism. The synthesis and secretion of TSH is stimulated by thyrotropin-releasing hormone (TRH), which is produced by the hypothalamus in response to low levels of circulating T3 and T4. In contrast, elevated levels of T3 and T4 suppress the production of TSH. Collectively, this negative feedback system is referred to as the hypothalamic-pituitary-thyroid axis. Any alteration in the function of this axis can influence the levels of TSH, T4, and T3 in circulation.
2. The principal clinical use for TSH measurement is for the assessment of thyroid status. TSH is measured in conjunction with thyroid hormones or antibodies to: 1) detect or exclude hypothyroidism or hyperthyroidism; 2) monitor T4 replacement treatment in hypothyroidism or antithyroid treatment in hyperthyroidism; 3) monitor TSH suppression in thyroid cancer patients on thyroxine therapy; and 4) assess the response to TRH stimulation testing.
3. Free T4 and free T3 regulate normal growth and development by maintaining body temperature and stimulating calorogenesis. In addition, free T4 and free T3 affect all aspects of carbohydrate metabolism as well as certain areas of lipid and vitamin metabolism. Fetal and neonatal development also requires thyroid hormone.
4. With normal levels of thyroid binding proteins, free T3 levels correlate with total T3. Measuring free T3 is useful when altered levels of total T3 occur due to changes in thyroid hormone binding proteins, especially in cases with altered TBG or low albumin concentrations. Free T3 is elevated alone (T3 toxicosis) in about 5% of hyperthyroids.
5. Clearly elevated free T4 levels support the clinical findings of a diagnosis of hyperthyroidism while clearly low free T4 levels coupled with appropriate clinical findings can establish a diagnosis of hypothyroidism. Measurement of free T4 levels along with other thyroid tests and clinical findings can establish borderline hyperthyroid and hypothyroid diagnoses.


Dr. Usha S.P. Yadav
M.D. Microbiology
ACHD/CH/NCR/PRYJ



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Nawab Yusuf Road

Prayagraj

Allahabad-211001,Uttar Pradesh, INDIA



Department of Laboratory Medicine

Serology

CR No : 991922200205615 Lab/Study No. : 0912SE101 Requisition Date : 12-Sep-2023 13:00
Patient Name : Sudha Srivastava Age/Sex : 64 Yr/F Coll./Study Date : 12-Sep-2023 13:00
Sample Type/No : Serum/1209W6009 Ward : Ward 4 Reporting Date : 12-Sep-2023 15:45
Room/Bed : Hdu/Hdu 6 Dept/Unit : General - GDMO R No 12

Investigation Result Unit Ref. Range

Dengue Virus (NS 1 IgM IgG) Rapid
Dengue Ns1 Negative
Dengue IgM Negative
Dengue Igg Negative

Comments:

***** END OF THE REPORT *****

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Allahabad-211001, Uttar Pradesh, INDIA

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Department of Laboratory Medicine

Hematology

CR No : 991922200205615 Lab/Study No. : 0912HM144 Requisition Date : 12-Sep-2023 13:00

Patient Name : Sudha Srivastava Age/Sex : 64 Yr/F Coll./Study Date : 12-Sep-2023 13:00

Sample Type/No : Edta Whole Blood/1209W6008 Ward : Ward 4 Reporting Date : 12-Sep-2023 14:05

Room/Bed : Hdu/Hdu 6 Dept/Unit : General - GDMO R No 12

Investigation	Result	Unit	Ref. Range
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Complete Blood Picture

Hemoglobin	11.2	g/dl	12 - 15
Hematocrit (PCV)	35.2	%	36 - 46
Mean Corpuscular Volume (MCV)	78.6	fl	83 - 101
Mean Corpuscular Hemoglobin (MCH)	25.0	pg	27 - 32
Mean Corpuscular Hemoglobin Concentration (MCHC)	31.8	g/dl	31.5 - 34.5
Total RBC Count	4.48	10 ⁶ /mm ³	3.8 - 4.8
RDW CV	14.6	%	11.6 - 14
RDW SD	40.7	fl	39 - 46
Total WBC Count	19400	/mm ³	4000 - 10000
Neutrophils	82.3	%	40 - 80
Lymphocytes	6.7	%	20 - 40
Monocytes	10.7	%	2 - 8
Eosinophils	0.2	%	1 - 6
Basophils	0.1	%	0 - 2
Platelet Count	3.20	Lac/mm ³	1.5 - 4

Comments:

***** END OF THE REPORT *****

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**Department of Microbiology & Pathology
Central Hospital, North Central Railway
Prayagraj.**

Lab Registration No. :
Name:- **SUDHA**
Ward :- FMW-11
CR No.:-991922200205615
Regd Date :-12-09-2023
Reporting Date:-12-09-2023
Age/Sex :- 64/F
Referred by :- Dr ANURAG

The test has been carried out in Fully Automated Immunoassay System-MINI-VIDAS using ELFA (Enzyme Linked Fluorescence Assay) technology.

a) SEPSIS

b) LOWER RESPIRATORY TRACT INFECTION

Dr. Usha S.P. Yadav
M.D. Microbiology
ACHD/CH/NCR/PRYJ



NORTH CENTRAL RAILWAY CENTRAL
HOSPITAL

Nawab Yusuf Road
-211001, UTTAR PRADESH



CR NO : 991922200205615

SAMPLE/ACCESSION NO : 0709W6006

REQUISITION DATE : 07-Sep-2023 12:58

NAME : Sudha Srivastava

GENDER/AGE : Female/64 Yr

COLLECTION DATE : 07-Sep-2023 13:44

STATUS : IPD

ACCEPTENCE DATE : 07-09-2023

RESULT ENTRY DATE : 07-Sep-2023 14:16

Parameter Name	Value	Bio. Ref. Range	Unit
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TEST NAME : Serum Electrolytes

Serum Sodium	128.3	136 mmol/L - 146 mmol/L	mmol/L
Serum Potassium	4.50	3.5 mmol/L - 5.1 mmol/L	mmol/L
Serum Chloride	91.3	98 mmol/L - 106 mmol/L	mmol/L

TEST NAME : Liver Function Test

Aspartate Aminotransferase (AST)	48.63	< 31 U/L	U/L
Alanine Aminotransferase (ALT)	91.26	< 34 U/L	U/L
Alkaline Phosphatase (ALP)	81.52	35 U/L - 105 U/L	U/L
Serum Albumin	3.93	3.5 g/dl - 5 g/dl	g/dl
Serum Total Protein	7.41	6.4 g/dl - 8.3 g/dl	g/dl
Conjugated Bilirubin	0.23	<= 0.2 mg/dl	mg/dl
Total Bilirubin	1.12	0.1 mg/dl - 1.2 mg/dl	mg/dl

TEST NAME : Renal Function Test

Serum Creatinine	1.47	0.6 mg/dl - 1.2 mg/dl	mg/dl
Serum Urea	103.98	15 mg/dl - 40 mg/dl	mg/dl
Uric Acid	7.82	--	--

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Department of Laboratory Medicine

Biochemistry

CR No : 991922200205615 Lab/Study No. : 0914BI154 Requisition Date : 14-Sep-2023 11:59
Patient Name : Sudha Srivastava Age/Sex : 64 Yr/F Coll./Study Date : 14-Sep-2023 12:15
Sample Type/No : Serum/1409W6009 Ward : Ward 4 Reporting Date : 14-Sep-2023 15:29
Room/Bed : Hdu/Hdu 6 Dept/Unit : General - GDMO R No 12

Investigation	Result	Unit	Ref. Range
RENAL FUNCTION TEST:			
Uric Acid	4.92	mg/dl	0.6 - 1.2
Serum Creatinine	1.33	mg/dl	15 - 40
Serum Urea	55.13		
Liver Function Test			
Total Bilirubin	1.83	mg/dl	0.1 - 1.2
Conjugated Bilirubin	0.46	mg/dl	<= 0.2
Aspartate Aminotransferase (AST)	21.83	U/L	< 31
Alanine Aminotransferase (ALT)	27.12	U/L	< 34
Alkaline Phosphatase(ALP)	52.87	U/L	35 - 105
Serum Total Protein	5.15	g/dl	6.4 - 8.3
Serum Albumin	3.02	g/dl	3.5 - 5

Comments: ***** END OF THE REPORT *****



NORTH CENTRAL RAILWAY CENTRAL HOSPITAL

Nawab Yusuf Road

Prayagraj

Allahabad-211001, Uttar Pradesh, INDIA

Department of Laboratory Medicine

FMM



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Hematology

CR No : 991922200205615 Lab/Study No. : 0914HM128 Requisition Date : 14-Sep-2023 11:59

Patient Name : Sudha Srivastava Age/Sex : 64 Yr/F Coll./Study Date : 14-Sep-2023 12:15

Sample Type/No : Edta Whole Blood/1409W6010 Ward : Ward 4 Reporting Date : 14-Sep-2023 14:45

Room/Bed : Hdu/Hdu 6 Dept/Unit : General - GDMO R No 12

Investigation	Result	Unit	Ref. Range
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Complete Blood Picture

Hemoglobin	10.3	g/dl	12 - 15
Hematocrit (PCV)	33.3	%	36 - 46
Mean Corpuscular Volume (MCV)	79.5	fl	83 - 101
Mean Corpuscular Hemoglobin (MCH)	24.7	pg	27 - 32
Mean Corpuscular Hemoglobin Concentration (MCHC)	31.1	g/dl	31.5 - 34.5
Total RBC Count	4.19	$10^6/\text{mm}^3$	3.8 - 4.8
RDW CV	14.8	%	11.6 - 14
RDW SD	41.6	fl	39 - 46
Total WBC Count	16500 78.6	$/\text{mm}^3$	4000 - 10000
Neutrophils	11.2	%	40 - 80
Lymphocytes	9.5	%	20 - 40
Monocytes	0.7	%	2 - 8
Eosinophils	0.0	%	1 - 6
Basophils	0.0	%	0 - 2
Platelet Count	2.38	Lac/ mm^3	1.5 - 4

Comments:

***** END OF THE REPORT *****

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Prayagraj
Allahabad-211001, Uttar Pradesh, INDIA
Department of Laboratory Medicine



Biochemistry

Lab No	: 991922200205615	Lab/Study No.	: 0916BI193	Requisition Date	: 16-Sep-2023 13:36
Patient Name	: Sudha Srivastava	Age/Sex	: 64 Yr/F	Coll./Study Date	: 16-Sep-2023 13:59
Sample Type/No	: Serum/1609W6025	Ward	: Ward 4	Reporting Date	: 16-Sep-2023 16:22
Room/Bed	: Hdu/Hdu 6	Dept/Unit	: General - GDMO R No 12		
Investigation	Result		Unit	Ref. Range	

Liver Function Test

Total Bilirubin	1.29	mg/dl	0.1 - 1.2
Conjugated Bilirubin	0.22	mg/dl	<= 0.2
Aspartate Aminotransferase (AST)	20.00	U/L	< 31
Alanine Aminotransferase (ALT)	12.45	U/L	< 34
Alkaline Phosphatase (ALP)	56.56	U/L	35 - 105
Serum Total Protein	4.67	g/dl	6.4 - 8.3
Serum Albumin	2.80	g/dl	3.5 - 5

RENAL FUNCTION TEST:

Uric Acid	4.88	mg/dl	0.6 - 1.2
Serum Creatinine	1.31	mg/dl	15 - 40
Serum Urea	46.96	mg/dl	
Serum Electrolytes			
Serum Sodium	127.6	mmol/L	136 - 146
Serum Potassium	2.63	mmol/L	3.5 - 5.1
Serum Chloride	92.0	mmol/L	98 - 106

Comments:

***** END OF THE REPORT *****



NORTH CENTRAL RAILWAY CENTRAL HOSPITAL

Nawab Yusuf Road

OPD Prescription

Name	Sudha Srivastava	CR No.	991922200205615
Age/Gender	64 Yr/F	Patient Category	PD
Father/Spouse Name	Not Available	Department(Unit/Consultant)	Casualty/Casualty
Visit Date	17/09/2023	Unit Head	
Mobile No.	9005820617	Station	ALLAHABAD DIVISION
UMID	33221735280B	Designation	

CHIEF COMPLAINT : DM2 MILD AKI RESOLVED COPD --LRTI RESOLVED CAME IN LOW SPO2 AND LEUCOCYTOSIS,

DIAGNOSIS : Essential (primary) hypertension(*Provisional, I10,)* Pre-existing type 2 diabetes mellitus(*Provisional, O24.1,)* Other chronic obstructive pulmonary disease(*Provisional, J44,)*

Rx/Drugs

01. ETOPHYLLINE 115 MG, THEOPHYLLINE 35 MG RETARD 150 MG ORAL TAB/CAP, single Tab, BD(1-0-1), 10 Days, Qty : 20, 17-09-2023
02. MONTELUKAST 10 MG AND LEVOCETIRIZINE 5 MG ORAL TABLET, Single Tab, OD(1-0-0), 10 Days, Qty : 10, 17-09-2023
03. AMBROXOL BP, TERBUTALINE SULFATE AND GUAIPHENESIN IP SYRUP 60 ML, One Tsp. BD(1-0-1), 10 Days, Qty : 1, 17-09-2023
04. PANTOPRAZOLE 40 MG ORAL TABLET, single Tab, OD(1-0-0), 10 Days, Qty : 10, 17-09-2023
05. CEFUROXIME 500 MG ORAL TABLET, single Tab, BD(1-0-1), 10 Days, Qty : 20, 17-09-2023
06. ITRACONAZOLE 100 MG ORAL TAB/CAP, Single Tab, OD(1-0-0), 10 Days, Qty : 10, 17-09-2023
07. TORSEMIDE 10 MG ORAL TABLET, single Tab, BD(1-0-1), 10 Days, Qty : 20, 17-09-2023
08. SALBUTAMOL 100MCG 200 MDI INHALER, Spl Instruction, BD(1-0-1), 10 Days, Qty : 1, 17-09-2023
09. SALMETEROL 25 MCG FLUTICASONE PROPIONATE 250 MCG INHALER, Spl Instruction, BD(1-0-1), 10 Days, Qty : 1, 17-09-2023
10. VITAMIN B COMPLEX FORTE AND ZINC ORAL CAPSULE, Single Cap, OD(1-0-0), 10 Days, Qty : 10, 17-09-2023
11. IRON FOLIC ACID AND VITAMIN B12 ORAL CAPSULES, single Cap, OD(1-0-0), 10 Days, Qty : 10, 17-09-2023

No Known Drug Allergy

FOLLOW UP : SOS

Signature of Consultant Resident :

Dr. Anurag Yadav (ADDL CHIEF HEALTH DIRECTOR)

17/9/2023

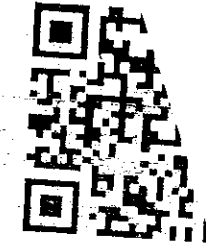
अपर मुख्य स्वास्थ्य निदेशक

डॉ० अनुराग यादव

Dr. ANURAG YADAV

Additional Chief Health Director

North Central Railway Prayagraj



Dr. Anurag
Gupta MD
DOPD
(Kidney doc)

17/09/23
7 AM.

(20)

, 84, 20, 109/64

FBS - 56 mg/dl

7:50 AM Repeat - 166 mg/dl

(11)

⑪

12/9
6pm BP 102/62 P-98

RBS
(193) HIR unit
Lantus 10 unit

13/9/23

BP PR

RBS

2am 108/65 92

(94)

8pm 90/60 208 mg/dl HR 2 unit
Lantus 10 unit

14/9
2am

BP PR

112/66 100

(99)

8pm 194 mg/dl HR 2 unit
Lantus 10 unit

15/9
7:30am

BP

PR

RBS

119/78 93

(114)

15/9/23 7pm — RBS — (221) HR 4 unit
Lantus 10

16/09/23 7pm 100, 80, 102/60, 95 mg/dl FBS - 73 mg/dl Insulin

1pm 86 20 106/63 95 mg/dl HR 10 unit



CENTRAL HOSPITAL
DEPARTMENT OF MICROBIOLOGY/MOLECULAR LAB
NORTH CENTRAL RAILWAY PRAYAGRAJ

Patient Name : SUDHA SRIVASTAVA	Received on : 11-09-2023
Age & Sex : 64/F	Reported on : 11-09-2023
Patient ID : 991922200205615	Ref. Doctor : Dr. Anurag Yadav
OPD/Ward : HDU-6	Serial No. : 01/1109

Result	Test Name	Result	Units	Unicel Dxl / Access 2	
				Reference Ranges	
				Range:	2.5 - 3.9
	FT3 (Free Triiodo thyronine)	2.96	pg/mL	Range:	0.61 - 1.12
				Pregnancy: 1st Trimester	0.52 - 1.10
				Pregnancy: 2nd Trimester	0.45 - 0.99
				Pregnancy: 3rd Trimester	0.48 - 0.95
	FT4 (Free Thyroxine)	1.58	ng/dl	Range:	0.34 - 5.6

TSH (Thyroid Stimulating Hormone) 1.721 μ U/mL

The test has been carried out in Automated ELIA System- Beckman Coulter Access-2.

Interpretation			
TSH	FT3	FT4	Interpretation
High	Normal	Normal	Subclinical Hypothyroidism
Low	Normal	Normal	Subclinical Hyperthyroidism
High	High	High	Secondary Hyperthyroidism
Low	High/Normal	High/Normal	Hyperthyroidism
Low	Low	Low	Non Thyroidal illness/Secondary Hypothyroidism

Comments

1. TSH, released from the anterior pituitary, is the principal regulator of thyroid function, stimulating the synthesis and release of thyroid hormones thyroxine (T4) and triiodothyronine (T3). T3 and T4 regulate biochemical processes that are essential for normal metabolism. The synthesis and secretion of TSH is stimulated by thyrotropin-releasing hormone (TRH), which is produced by the hypothalamus in response to low levels of circulating T3 and T4. In contrast, elevated levels of T3 and T4 suppress the production of TSH. Collectively, this negative feedback system is referred to as the hypothalamic-pituitary-thyroid axis. Any alteration in the function of this axis can influence the levels of TSH, T4, and T3 in circulation.
2. The principal clinical use for TSH measurement is for the assessment of thyroid status. TSH is measured in conjunction with thyroid hormones or antibodies to: 1) detect or exclude hypothyroidism or hyperthyroidism; 2) monitor T4 replacement treatment in hypothyroidism or antithyroid treatment in hyperthyroidism; 3) monitor TSH suppression in thyroid cancer patients on thyroxine therapy; and 4) assess the response to TRH stimulation testing.
3. Free T4 and free T3 regulate normal growth and development by maintaining body temperature and stimulating calorogenesis. In addition, free T4 and free T3 affect all aspects of carbohydrate metabolism as well as certain areas of lipid and vitamin metabolism. Fetal and neonatal development also requires thyroid hormone.
4. With normal levels of thyroid binding proteins, free T3 levels correlate with total T3. Measuring free T3 is useful when altered levels of total T3 occur due to changes in thyroid hormone binding proteins, especially in cases with altered TBG or low albumin concentrations. Free T3 is elevated alone (T3 toxicosis) in about 5% of hyperthyroids.
5. Clearly elevated free T4 levels support the clinical findings of a diagnosis of hyperthyroidism while clearly low free T4 levels coupled with appropriate clinical findings can establish a diagnosis of hypothyroidism. Measurement of free T4 levels along with other thyroid tests and clinical findings can establish borderline hyperthyroid and hypothyroid diagnoses.


Dr. Usha S.P. Yadav
M.D. Microbiology
ACHD/CH/NCR/PRYJ



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Prayagraj

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Department of Laboratory Medicine

Biochemistry

CR No : 991922200205615 Lab/Study No. : 0911BI191 Requisition Date : 10-Sep-2023 10:46
Patient Name : Sudha Srivastava Age/Sex : 64 Y/F Coll./Study Date : 11-Sep-2023 13:19
Sample Type/No : Serum/1109W4009 Ward : IPD Reporting Date : 11-Sep-2023 18:17
Room/Bed : -/- Dept/Unit : General - Gdmo R No 12

Investigation	Result	Unit	Ref. Range
Serum Creatinine	1.53	mg/dL	0.7 - 1.2
Serum Uric Acid	6.08	mg/dL	2.6 - 6.0

Comments:

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Department of Pathology and Microbiology
Central Hospital, North Central Railway
Prayagraj

Date : 10-Sep-2023
Name : MRS. SUDHA SRIVASTAVA CR-991922200205615
Ref.By : ---
Coll. Time : Plain
Reg/Ref: I-148007 / 430468
Collected At : Indoor
Age/Sex : /Female

BIOCHEMISTRY

Renal / Kidney Profile (KFT/RFT)

Serum Creatinine
Serum Urea
Serum Uric Acid

1.03 mg/dL
81.62 mg/dL
5.44 mg/dL

0.8 TO 1.4
15 - 45
2.4 - 5.7

Electrolytes, Serum
Serum Sodium (Na+)
Serum Potassium (K+)
Serum Chlorides

129.4 meq/l
3.02 meq/l
86.4 mmol/L

132 - 145
3.5 - 5.0
98 - 109



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Department of Laboratory Medicine

Biochemistry

CR No : 991922200205615 Lab/Study No. : 0911B1191 Requisition Date : 10-Sep-2023 10:46
Patient Name : Sudha Srivastava Age/Sex : 64 Yr/F Coll./Study Date : 11-Sep-2023 13:19
Sample Type/No : Serum/1109W4009 Ward : IPD Reporting Date : 11-Sep-2023 18:17
Room/Bed : -/- Dept/Unit : General - Gdmo R No 12

Investigation	Result	Unit	Ref. Range
Renal Function Test			
Serum Creatinine	1.53	mg/dL	0.7 - 1.2
Serum Uric Acid	6.08	mg/dL	2.6 - 6.0

Comments:

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Department of Laboratory Medicine

Biochemistry

CR No : 991922200205615 Lab/Study No. : 0907BI008 Requisition Date : 07-Sep-2023 10:31
Patient Name : Sudha Srivastava Age/Sex : 64 Yr/F Coll./Study Date : 07-Sep-2023 10:38
Sample Type/No : Serum/0709W4005 Ward : Ward 4 Reporting Date : 07-Sep-2023 12:49
Room/Bed : Hdu/Hdu 6 Dept/Unit : General - GDMO R No 12

Investigation	Result	Unit	Ref. Range
Lipid Profile:			
Serum Cholesterol	175.80	mg/dl	Desirable < 200; Boderline - 200-239; High > 239
Serum Triglycerides	76.43	mg/dl	Normal: < 150; High: 150-199; Hyper triglyceridemia:200-499; Very High > 499
Serum HDL Cholesterol	41.38	mg/dl	> 40 mg/dl
Serum LDL Cholesterol		mg/dl	Optimal: < 100; Near/above Optimal: 100-129; Borderline : 130-159; High 160-189; Very High: > 189 mg/dl

Comments:

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Department of Laboratory Medicine



Biochemistry

CR No : 991922200205615
Patient Name : Sudha Srivastava
Sample Type/No : Serum/0709W6006
Room/Bed : Hdu/Hdu 6
Investigation :
Lab/Study No. : 0907BI022
Age/Sex : 64 Yr/F
Ward : Ward 4
Dept/Unit : General - GDMO R No 12
Requisition Date : 07-Sep-2023 12:58
Coll./Study Date : 07-Sep-2023 13:44
Reporting Date : 07-Sep-2023 14:19
Unit

Result

RENAL FUNCTION TEST:

Uric Acid					
Serum Creatinine	7.62	mg/dl	0.6 - 1.2		
Serum Urea	1.47	mg/dl	15 - 40		
Serum Electrolytes	103.98				
Serum Sodium		mmol/L	136 - 146		
Serum Potassium	128.3	mmol/L	3.5 - 5.1		
Serum Chloride	4.50	mmol/L	98 - 106		
Liver Function Test	91.3				
Total Bilirubin	1.12	mg/dl	0.1 - 1.2		
Conjugated Bilirubin	0.23	mg/dl	<= 0.2		
Aspartate Aminotransferase (AST)	48.63	U/L	< 31		
Alanine Aminotransferase (ALT)	91.26	U/L	< 34		
Alkaline Phosphatase(ALP)	81.52	U/L	35 - 105		
Serum Total Protein	7.41	g/dl	6.4 - 8.3		
Serum Albumin	3.93	g/dl	3.5 - 5		

Comments:

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Department of Laboratory Medicine

Biochemistry

CR No : 991922200205615 Lab/Study No. : 0909BI162 Requisition Date : 09-Sep-2023 10:38
Patient Name : Sudha Srivastava Age/Sex : 64 Yr/F Coll./Study Date : 09-Sep-2023 11:56
Sample Type/No : Serum/0909W4004 Ward : Ward 4 Reporting Date : 09-Sep-2023 14:36
Room/Bed : Hdu/Hdu 6 Dept/Unit : General - GDMO R No 12

Investigation	Result	Unit	Ref. Range
Liver Function Test			
Total Bilirubin	1.38	mg/dl	0.1 - 1.2
Conjugated Bilirubin	0.52	mg/dl	<= 0.2
Aspartate Aminotransferase (AST)	27.50	U/L	< 31
Alanine Aminotransferase (ALT)	60.19	U/L	< 34
Alkaline Phosphatase (ALP)	72.71	U/L	35 - 105
Serum Total Protein	6.34	g/dl	6.4 - 8.3
Serum Albumin	3.60	g/dl	3.5 - 5

Comments:

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Ward :
Scribe :

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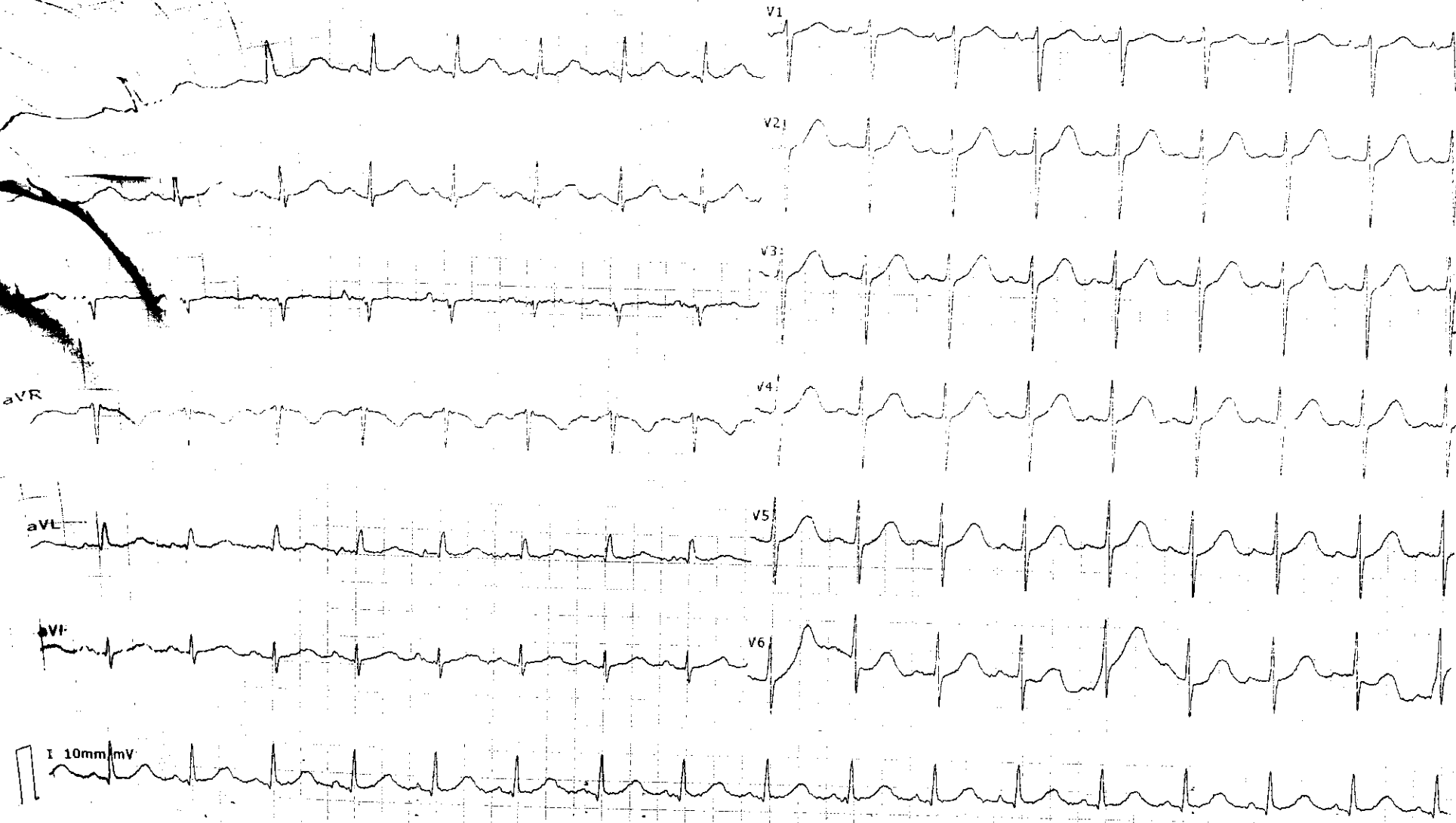
Gender: _____
Height(cm): _____
BP(mmHg): /

P-R.....ms 183
Q-R-S.....ms 93
QT/QTc.....ms 390/503
P/QRS/T AXES.....deg 52/1/52
RV5/SV1.....mV 0.76/0.86
RV5+SV1.....mV 1.62

<< Conclusion >>
803 Sinus Tachycardia
** SUSPECTED ABNORMAL ECG **

Report Confirmed by: _____

*The result must be confirmed by doctor!





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-211001, UTTAR PRADESH



CR NO: 591922200205615

REQUISITION DATE : 05-Sep-2023 10:28

GENDER/AGE : Female/64 Yr

STATUS : IPD

RESULT ENTRY DATE : 06-Sep-2023 10:01

SAMPLE/ACCESSION NO : 0509OP180

NAME : Sudha Srivastava

COLLECTION DATE : 05-Sep-2023 10:28

ACCEPTANCE DATE : 05-09-2023

Parameter Name	Value	Bio. Ref. Range	Unit
TEST NAME : RT PCR for SARS nCoV2			
SARS CoV-2 (COVID-19)	4.00E+06	--	--
Condition of Specimen	Not a Suitable Cold Chain	--	--
covid sample id		--	--
ICMR Lab ID		--	--

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Department of Laboratory Medicine

Serology

CR No	: 991922200205615	Lab/Study No.	: 0905SE004	Requisition Date	: 05-Sep-2023 10:28
Patient Name	: Sudha Srivastava	Age/Sex	: 64 Yr/F	Coll./Study Date	: 05-Sep-2023 10:28
Sample Type/No	: Nps/0509OP179	Ward	: Ward 6	Reporting Date	: 06-Sep-2023 13:37
Room/Bed	: Female Medical Ward/Fmw 8	Dept/Unit	: General - GDMO R No 20		
Investigation SARS CoV-2 Antigen	Result	Unit	Ref. Range		

Comments:

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Department of Laboratory Medicine

Biochemistry

CR No : 991922200205615 Lab/Study No. : 0906BI139 Requisition Date : 06-Sep-2023 10:47
Patient Name : Sudha Srivastava Age/Sex : 64 Yr/F Coll./Study Date : 06-Sep-2023 11:54
Sample Type/No : Plasma/0609W4009 Ward : Ward 4 Reporting Date : 06-Sep-2023 15:14
Room/Bed : Hdu/Hdu 6 Dept/Unit : General - GDMO R No 12
Investigation Result Unit Ref. Range

Fasting plasma Glucose

85.81

mg/dl

Normal 70 - 100 mg/dl;
Impaired Fasting Glucose
101-125 mg/dl; Diabetes
>126 mg/dl

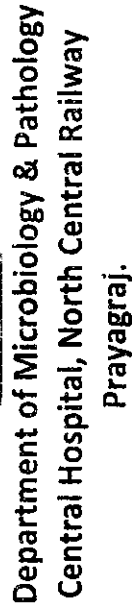
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a) SEPSIS

PCT Reference Range (ng/ml)	<0.5 ng/ml	≥0.5 - <2.0ng/ml	≥2.0 - <10ng/ml	≥ 10ng/ml
Interpretation of Systemic Bacterial Infection	UNLIKELY	POSSIBLE	LIKELY	VERY LIKELY
Progression to Severe Sepsis and/or Septic Shock	LOW RISK	MODERATE RISK	HIGH RISK	VERY HIGH RISK

PCT Reference Range (ng/ml)	<0.1 ng/ml	0.1 - 0.25ng/ml	>0.25 - 0.5ng/ml	>0.5ng/ml
Interpretation of Bacterial Etiology	VERY UNLIKELY	UNLIKELY	LIKELY	VERY LIKELY
Recommendation on Antibiotic therapy	STRONGLY DISCOURAGED	DISCOURAGED	RECOMMENDED	STRONGLY RECOMMENDED

CLC
CH/NCR/PRY



Department of Microbiology & Pathology
Central Hospital, North Central Railway
Prayagraj.

Lab Registration No. Regd Date :-06-09-2023
Name:- SUDHA SRIVASTAVA Reporting Date:- 06-09-2023
Ward :- HDU-6 Age/Sex :- 64/M
CR No:- 991922200205615 Referred by :- Dr ANURAG

Dengue Test Report – Automated testing

TEST	RESULT	Unit	Result Interpretation
Dengue NS1	NEGATIVE	0.10	< 1.0 (Index Value) NEGATIVE
Dengue Ig M	NEGATIVE	0.04	≥ 1.0 (Index Value) POSITIVE

Method: Test performed with automated testing on ELFA technology (miniVIDAS/VIDAS) –
Interpretation of results in different stage of Infection :

Dengue infection stage	VIDAS® DENGUE NS1 Ag	VIDAS® Anti-DENGUE IgM	VIDAS® Anti-DENGUE IgG
Naive	Negative	Negative	Negative
Acute infection	Positive	Positive	Negative
Post-acute infection	Negative	Positive	Positive
Recovery	Negative	Negative	Positive

Notes:

- Dengue virus (DENV) belongs to the Flaviviridae family and is transmitted to humans by Aedes mosquitoes. DENV are divided into four serotypes: DEN-1, DEN-2, DEN-3 and DEN-4. The test detects all 4 serotypes.
- Recommended test is NS1 Antigen in the first 5 days of fever. After 7-10 days offever, the recommended test is Dengue fever antibodies IgG & IgM.
- Negative IgM result does not rule out Dengue infection. Additional sample to be tested after 7-14 days if infection is suspected, and if positive Suggestive of Primary / Secondary Dengue infection.
- Negative IgG indicates a presumptive evidence that the patient does not have secondary Dengue infection and if positive indicating presumptive evidence that the patient has been recently exposed to / or currently infected with Dengue virus
- In presence of detectable dengue NS1 antigen, Dengue IgG & IgM serology assays should be performed on follow up samples after 5-7 days of onset of fever, to confirm dengue infection.
- The disease may be subclinical, self limiting, febrile or may progress to a severe form of Dengue hemorrhagic fever or Dengue shock syndrome.
- Interference may be encountered with certain sera containing antibodies directed against reagent components. For this reason, assay results should be interpreted taking into consideration the patient's clinical history and the results of any other tests performed

1305/2023/5/2023
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Department of Laboratory Medicine

Hematology

CR No : 991922200205615 Lab/Study No. : 0907HM034 Requisition Date : 07-Sep-2023 12:58
Patient Name : Sudha Srivastava Age/Sex : 64 Yr/F Coll./Study Date : 07-Sep-2023 13:44
Sample Type/No : Edta Whole Blood/0709W6007 Ward : Ward 4 Reporting Date : 07-Sep-2023 13:59
Room/Bed : Hdu/Hdu 6 Dept/Unit : General - GDMO R No 12
Investigation Result Unit Ref. Range

Complete Blood Picture

Hemoglobin	11.2	g/dl	12 - 15
Hematocrit (PCV)	35.8	%	36 - 46
Mean Corpuscular Volume (MCV)	79.3	fl	83 - 101
Mean Corpuscular Hemoglobin (MCH)	24.9	pg	27 - 32
Mean Corpuscular Hemoglobin Concentration (MCHC)	31.4	g/dl	31.5 - 34.5
Total RBC Count	4.51	10 ⁹ /mm ³	3.8 - 4.8
RDW CV	14.3	%	11.6 - 14
RDW SD	40.3	fl	39 - 46
Total WBC Count	13900	/mm ³	4000 - 10000
Neutrophils	87.4	%	40 - 80
Lymphocytes	6.2	%	20 - 40
Monocytes	6.2	%	2 - 8
Eosinophils	0.0	%	1 - 6
Basophils	0.2	%	0 - 2
Platelet Count	4.47	Lac/mm ³	1.5 - 4

Comments:

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Department of Laboratory Medicine

Biochemistry

CR No : 991922200205615 Lab/Study No. : 0907BI022 Requisition Date : 07-Sep-2023 12:58
Patient Name : Sudha Srivastava Age/Sex : 64 Yr/F Coll./Study Date : 07-Sep-2023 13:44
Sample Type/No : Serum/0709W6006 Ward : Ward 4 Reporting Date : 07-Sep-2023 14:19
Room/Bed : Hdu/Hdu 6 Dept/Unit : General - GDMO R No 12
Investigation Result Unit Ref. Range

RENAL FUNCTION TEST:

Uric Acid 7.82 mg/dl 0.6 - 1.2
Serum Creatinine 1.47 mg/dl 15 - 40
Serum Urea 103.98

Serum Electrolytes

Serum Sodium 128.3 mmol/L 136 - 146
Serum Potassium 4.50 mmol/L 3.5 - 5.1
Serum Chloride 91.3 mmol/L 98 - 106

Liver Function Test

Total Bilirubin 1.12 mg/dl 0.1 - 1.2
Conjugated Bilirubin 0.23 mg/dl <= 0.2

Aspartate Aminotransferase 48.63

(AST) U/L < 31

Alanine Aminotransferase 91.26

(ALT) U/L < 34

Alkaline Phosphatase(ALP) 81.52

U/L 35 - 105

Serum Total Protein 7.41

g/dl 6.4 - 8.3

Serum Albumin 3.93

g/dl 3.5 - 5

Comments:

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Department of Laboratory Medicine

Serology

CR No : 991922200205615 Lab/Study No. : 0912SE101 Requisition Date : 12-Sep-2023 13:00
Patient Name : Sudha Srivastava Age/Sex : 64 Yr/F Coll./Study Date : 12-Sep-2023 13:00
Sample Type/No : Serum/1209W6009 Ward : Ward 4 Reporting Date : 12-Sep-2023 15:45
Room/Bed : Hdu/Hdu 6 Dept/Unit : General - GDMO R No 12

Ref. Range

Unit

Result

Dengue Virus (NS 1 IgM IgG) Rapid
Dengue Ns1 Negative
Dengue IgM Negative
Dengue IgG Negative

Comments:

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